طبع فى 19:48:55 11/10/2026

جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ابتسام انور كامل وهبه
المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** -
رقم البطاقة : 27505172401127 **رقم التليفون :** 01111216983 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 17-05-1975 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ابتسام انور كامل وهبه
National ID: 27505172401127

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

06/28/2026

MDT FORM

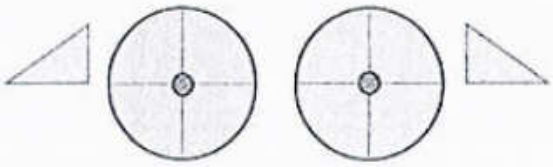
Name:	Ebtesam Anwar Kamal .	**MRN:**	842
Age:	51 Y	**ID:**	27505172401127
Menopausal Sta		**Phone:**	01220205873
NRS SCORE:	ps 1-2	**Marital Status:**	
BMI:		**Family History:**	-ve
CO Morbidity:	type 1 diabetes	**Reason Of MDT:**	dosease progressopn

Clinical Data:

BX: denovo metastatic breast cancer to bone with new liver m

ER: 0/8 **PR:** 3/8 **HER2:** +3 **KI67:**

R L



TNM

Sonomammography:

Mets Workup:

History:
denovo MET breast cancer to bone, received 33 weeks of taxol last 16/5/26
with herceptin
last CT SCAN 11/2 sd (mets to bones only)
bone scan sd
patient was prescibed Xeloda tykerb however no evidence of progression
so continued on taxol herceptin
recent CTs on 10 /6 showed stationary disease in breast
progressive course right axillary lns 27x25 (prev 25x14) mm
newly developed hfls largest 17x16mm
small uterine fibroids

bone scan pending

Recommendations:

MDT Outcome & Plan
for xeloda tykerb

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ هنان عزت	د/				

Patient's Name: Ibtisam Anwar Kamel
Date: 2026-06-10
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

No available clinical data.

I- CHEST:

- Right breast lesion measuring 33 x 28 mm with skin thickening and right axillary lymph node measuring 27 x 25 mm for sono-mammography.
- Few tiny bilateral scattered pulmonary nodules measuring 2-3 mm in diameter.
- Multiple bilateral scattered atelectatic bands.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. Few bilobar scattered homogenously enhancing focal lesions largest measuring 17 x 16 mm at segment IV. No dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Small uterine focal lesion is seen measuring 20 x 17 mm showing marginal contrast uptake likely fibroid for sonographic assessment.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Heterogenous CT density of L2 vertebral body with mild vertebral wedging.

Opinion:

Name: ابتسام انور كامل	
MR. : 842	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

Bone scan on 12/25 showed new lesions at the 7th rib and left acetabulum; MDT on 1/26 recommended shifting to Tykerb + Xeloda.

MDT in March 2026 continued W18 TAXWOL 130 mg + C7 HERCEPTIN 440 mg (WT 80 HT 160 SA 1.8); W19 and W20 were in PH.

Pharmacy visit for week 19 Taxol 130 mg; labs on 16/3 accepted.

Pharmacy visit for week 21 Taxol 130 mg + C8 HERCEPTIN 440 mg; labs on 16/3 accepted.

Pharmacy visit for week 22 Taxol 130 mg; labs on 7/4 accepted.

Prem-RT breast cancer CT3 N2 M1 initial BS L2 lesion ER-ve PR3/8 HER23+; REC Taxol HERCEPTIN 11/2026 BS new sites left acetabulum, 7th ribs, and L2. Stopped TT due to MDT for Xeloda Tykerb; stopped 2 m due to approval. MDT 3/2026 to resume Taxol HERCEPTIN until new CTs for W23; Taxol 140 mg grade 2 PN; HERCEPTIN next week for dental exam for Zometa and new CTs.

Patient has severe pain and left upper limb weakness for MRI brain; pall RTH dental exam for W24; Taxol 140 mg, C9 HERCEPTIN IV and liver support ALT 70.

Still mild pain related to left lower limb restricted movement; left lower limb venous duplex MRI brain pending; pall RTH pending for W25,26 Taxol 140 mg (BS 2/2026 left acetabulum L2 left 7th rib).

Last lab 28/4 accepted; new lab next cycle EF 63% 16/3/2026 for W27 Taxol 140 mg; 12/5 C10 HERCEPTIN IV.

CTs 11/2/2026 free BS 9/2 SD of old osseous lesions LV2 left 7th rib left acetabulum. On exam right breast edema redness; new CTs for W28 115 Taxol (20 Dr due to grade 2 PN); C10 HERCEPTIN 440.

On exam right breast edema; CTs pending for W29 Taxol 115 W30 Taxol; patient is on Cymbatex.

Patient ended palliative radiotherapy to L2 left hip joint 30 Gy/10 fractions from 20/5/2026 to 3/6/2026. On exam right breast edema redness; right axillary lymph nodes 3 × 3 cm; new US breast for new CTs; bone scan meanwhile for C31 Taxol 115 and C11 HERCEPTIN 440 and Zometa for W32 PH for CBC next week.

تحريرا في ,

التاريخ :	20-09-2026	الوقت :	

الطبيب المعالج : dina makram

يعتمد ,

Denovo MET breast cancer to bone; received 33 weeks of Taxol last 16/5/26 with HERCEPTIN last CT scan 11/2 SD (mets to bones only). Bone scan SD patient was prescribed Xeloda Tykerb; however no evidence of progression so continued on Taxol HERCEPTIN. Recent CTs on 10/6 showed stationary disease in breast; progressive course right axillary lymph nodes 27 × 25 mm (prev 25 × 14 mm); newly developed HFLs largest 17 × 16 mm; small uterine fibroids echo EF 63% for W32 Taxol today W33 pharmacy.

MDT on 28/6/2026; for Xeloda Tykerb for C12 HERCEPTIN 440 IV until approval of Xeloda Tykerb.

MDT 6/2026 for Xeloda + Tykerb; next visit 8/8/2026.

Today for C2 Xeloda 3 × 2 × 14 lab >>> OK; patient complained of dysphagia, diarrhea G11 Tykerb (17-8-2026) >> Pharmacy.

MDT 6/2026 for Xeloda + Tykerb C3 Xeloda + Tykerb; next visit 20/9/2026.

The patient has completed 3 cycles of Xeloda + Tykerb.

ıent's Name:	Ibtisam Anwar Kamel
ıte:	2026-06-10
ferred by:	Prof.Dr. no doctor
ospital:	Goustave Roussey International Egypt

- Right breast lesion with axillary lymph node for sono-mammography.
- Few tiny bilateral scattered pulmonary nodules ? inflammatory for follow up.
- Hepatomegaly showing few bilobar hepatic focal lesions for dynamic MR study.
- Heterogenous CT density of L2 vertebral body showing mild wedging for bone scan.

Dr.sherif Wagdy

Dr.sherif Wagdy

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمى
The General Organization for Teaching Hospitals and Institutes
Ahmad Maher Teaching Hospital

To whom it may concern

Mrs./ Mr ..... years old

female/ male patient presented with diagnosis .....

.....MBC.....

She/ He was referred to us for (radical / adjuvant / Neoadjuvant / palliative) Radiotherapy

She/ He has received (3D/ IMRT) radiotherapy to.....

.....L2.....&..Lt.hip.....

.....

At Dose : ..30.Gy/.10.fx/..2..w .

Additional (Boost) ; yes/ no, at dose .....Gy/.....fx/.....w

(sequential / concomitant)

Started: 29/.5./2026.

Ended: 3 /.6./2026 With good tolerance .

Head of department
Dr. Hany William

E-mail: Amaher.gothi.com 23914455: ت -23911838- 23913366 : ت 3411شارع بور سعيد – باب الخلق – القاهرة


جمهورية مصر العربية
بطاقة تحقيق الشخصية
ابتسام
انور كامل وهبه
١٠ ش التوحيد - المنشية
الهرم - الجيزه
٢٧ ١١ ٠ ٢٤ ١٧ ٠٥ ٢٧٥
JV2671536



GUSTAVE ROUSSY INTERNATIONAL EGYPT

Name : ebtesam anwer kamil	**Date : 11-02-2026 09:32**
Branch : Portable	**Medical No. : GRI-110652**
Ref.Physician Prof : cap+c	**Ins.Company :**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Compared to previous study dated 13-11-2025

I- CHEST:

- Still noted right breast ILQ speculated lesion measures about 2.7x3 cm with skin retraction associated with skin thickness measures about 7 mm.
- Still noted multiple enlarged right axillary lymph nodes with thickened cortex, the largest is seen measuring about 2x1.4 cm with central necrosis.
- Still noted bilateral few scattered subcentimetric nodules with the largest is seen in right upper lobe posterior segment measures about 4mm.
- Right middle lobe and lingual parenchymal bands are noted
- No consolidation or collapse.
- No pathological hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- No gross cardiomegaly

II- ABDOMEN AND PELVIS:

- The liver is enlarged in size showing homogenous fatty parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Normal C.T appearance of the spleen, pancreas, Both kidneys, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.

CT Scan
[[DOCTYPE:CT_SCAN]]

PATIENT NAME: EBTESAM ANWAR KAMEL WAHBA **GENDER:** Female **DATE :** 24-May-2025 12:33 PM	**PATIENT ID:** 10125262471 **AGE:** 50 y **ORDER:** 3383188

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

Active worrying bone lesion at LV2.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

Suspicious LV2 lesion.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant **24-May-2025 03:07 PM**

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16602
٩ شارع علوة أول شارع الهرم بجوار دارى الجيزة

PATIENT NAME: EBTESAM ANWAR KAMEL WAHBA
GENDER: Female
DATE : 13-May-2025 11:17 AM

PATIENT ID: 10125262471
AGE: 49 y
ORDER: 3373778,3375906

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIPHASIC STUDY OF THE LIVER

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion.

FINDINGS:

1- CHEST:

**** Patient with pathologically proven right breast IDC:**

- Right breast LIQ spiculated heterogeneously enhancing mass is seen, it measures about 2.8 x 2.5 cm along its maximum axial dimensions. Related diffuse thickening of the right breast skin is seen. A small subcutaneous nodular lesion is seen at a lower position to the lesion along the right aspect of the upper abdominal wall.
- Multiple pathological enlarged right axillary level I and smaller level II, III lymph nodes, the largest seen at in level I measuring about 2.9 x 2.4 cm along its maximum axial dimensions. The larger lymph nodes show heterogeneous enhancement with central hypodense non-enhancing areas (of break down).
- Few tiny bilateral internal mammary lymph nodes, measuring about 0.3 - 0.4 cm.
- Few prominent bilateral supraclavicular lymph nodes, the largest measuring about 0.7 x 0.8 cm.
- Few bilateral pulmonary nodules are seen, the largest is seen at the posterior segment of the right upper lung lobe measures about 0.5 cm.
- No pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.

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مركز بهية الهرم
4 شارع عزيزة أول شارع الهرم بجوار دارى ، الجيزة
مستشفى بهية الشيخ زايد
١ الحي الأول - الشيخ زايد أمام كلية الهندسة

- No pericardial effusion detected.
- Small hiatus hernia.

2- ABDOMEN AND PELVIS:

- The liver is of average size with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Normal appearance of the pancreas, spleen, kidneys, adrenals, aorta and I.V.C.
- The uterus shows a fundal lesion, likely fibroid, for U/S correlation.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic masses or collections.

The revised bone window revealed:

1. L2 vertebral body and left iliac bone lytic lesions, larger at the latter measures about 20 x 18 mm.
2. No CT evidence of vertebral collapse or extra-osseous soft tissue component.
3. Left clavicular and left femoral neck sclerotic foci, likely bone islands.
4. Central wedging of D5 and to less extent D4 vertebral bodies.

OPINION:

- **Right breast neoplastic process with enlarged ipsilateral axillary lymph nodes.**
- **Few tiny bilateral internal mammary and supraclavicular lymph nodes, for follow-up.**
- **Few bilateral pulmonary nodules, for close follow-up.**
- **No sizable hepatic metastatic deposits.**
- **Few osseous lytic lesions as described likely metastatic, for bone scan correlation.**

Reported By,
Dr. Sara Saif, MSc
Dr. Salwa Yahya, MD **14-May-2025 10:30 AM**

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مركز بهية الهرم
٩ شارع علوية أول شارع الهرم بجوار دارى , الجيزة
مستشفى بهية الشيخ زايد
٤٠ الحي الأول - الشيخ زايد أمام كلية الهندسة

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Ebtesam Anwar Kamel Wahba	Date : 16/12/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 50 Year(s)
Medical ID:	Pat. ID: 110652

FOLLOW UP BILATERAL BREAST HIGH RESOLUTION ULTRASOUND

INDICATION:
Right breast pathologically proven carcinoma, treated by NACT, for assessment

REPORT:

RIGHT BREAST:

- An ill-defined solid mass located at 4 o'clock is seen about 5.5cm from the nipple, measuring 35x27mm, associated with skin infiltration and thickening.
- Multiple pathological axillary nodes are also still seen in level I, reaching 24x20mm.

LEFT BREAST:

- No solid or cystic masses.
- Normal skin thickness, and nipple-areola complex.
- Nonspecific axillary nodes.

OPINION:

- Right breast locally advanced neoplastic process as described, (BIRADS 6).
- Left normal study, (BIRADS 1).

Best regards,
Dr. Mahi Mokhtar

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Ebtesam Anwar Kamel Wahba	Date : 13/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 50 Year(s)
Medical ID: 1424/2025	Pat. ID: 110652

- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Enlarged bilateral inguinal lymph nodes, the largest on left side measures about 1.6 cm
- No ascites.
- L2 vertebral body showed reduced height with sclerotic lesion.

Opinion:

- **Bilateral lung lobes tiny scattered nodules.**
- **Bilateral lung lobes atelectatic bands.**
- **Fatty hepatomegaly.**
- **Enlarged inguinal lymph nodes.**
- **L2 reduced height with sclerotic lesion ?? metastasis for further bone scan.**

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY

Signature Dr.Mohamed Hassan
16/11/2025 3:02 AM

CT Scan
[[DOCTYPE.CT_SCAN]]

PATIENT NAME: EBTESAM ANWAR KAMEL WAHBA
GENDER: Female
DATE : 29-Apr-2025 09:01 AM

PATIENT ID: 10125262471
AGE: 43 y
ORDER: [illegible]

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Right breast mass, no family history

TECHNIQUE:

- Complementary ultrasound

REPORT:

- Entirely fatty breasts [illegible]
- Right LIQ irregular spiculated hyperdense lesion is seen, right breast diffuse skin thickening reaching 8 mm, the lesion is seen by ultrasound as an irregular spiculated heterogeneous hypoechoic vertically oriented [illegible] on measuring about 2.4x3.5 cm showing [illegible]
- Right breast: No [illegible]
- No left spiculated mass lesions or grouped microcalcifications.
- Left UOQ superficial small circumscribed nodule measuring 5x3 mm [illegible]
- Normal left skin thickness and contour.
- Normal left nipple areola complexes.
- Multiple pathological enlarged right axillary level 1, 2 and 3 lymph nodes (collectively about 11 in number), the largest in level 1 measures about 17x16mm with focal cortical thickening of 12mm and effaced hilum. A right axillary tall irregular hypoechoic lesion is seen measuring about 2.3x2.1 cm with abnormal internal vascularity, possibly representing a pathological axillary lymph node with extracapsular rupture
- Free right supraclavicular region.
- Nonspecific left axillary lymph nodes.

OPINION:

- Right LIQ malignant looking lesion, with multiple pathological ipsilateral axillary level 1, 2 and 3 lymph nodes, for histopathological assessment. (BIRADS 5)
- Right UOQ focal asymmetry, for follow up.
- Benign looking nodule of the left breast (BIRADS 3).

RECOMMENDATION:

Referral to clinic

Medical No. : 3806
Patient's Name: Ibtisam Anwar Kamel
Date: 2026-09-30
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

No available clinical data.

I- CHEST:

- Right breast lesion measuring 32 x 17 mm with skin thickening and right axillary lymph node measuring 23 x 13 mm for sono-mammography.
- Few tiny bilateral scattered pulmonary nodules measuring 2-3 mm in diameter.
- Multiple bilateral scattered atelectatic bands.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. Small right hepatic lobe segment VIII enhancing focal lesion largest measuring 11 mm at in diameter. No dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Small uterine focal lesion is seen measuring 20 x 17 mm showing marginal contrast uptake likely fibroid for sonographic assessment.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Heterogenous CT density of L2 vertebral body with mild vertebral wedging.

Medical No. : 3806
Patient's Name: Ibtisam Anwar Kamel
Date: 2026-09-30
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- L4 down to S1 vertebral bodies sclerotic foci.

Opinion:

Compared to the previous study dated 10-6-2026, the current study showed:

- **Regressive course regarding the previously seen right breast lesion and axillary lymph node as well as the right and number of the previously seen bilobar hepatic focal lesions.**
- **Newly developed few L4 down to S1 vertebral bodies sclerotic foci for bone scan.**
- **Stationary course regarding the rest of the study.**

Dr.sherif Wagdy

Dr.sherif Wagdy